

Question:

Which of the following factors is associated with a decreased risk of Alzheimer's?

Answer Choices:

- A. Being African or Hispanic American
- B. A lower level of education
- C. Living in a city
- D. Being married
- E. Having a family history of Alzheimer's
- F. High intake of saturated fats
- G. Older age
- H. Eating fish
- I. Being a smoker
- J. Drinking alcohol regularly

Reasoning + Answer:

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Alzheimer’s disease shows clear sociodemographic and lifestyle risk-modifying patterns .

- Education : Higher educational attainment is consistently associated with reduced risk of Alzheimer’s pathology and clinically evident dementia , including APOE ε4 carriers . Cognitive reserve enhances resilience to neurodegenerative processes , especially late in life when pathologic burden increases . Thus , option A (‘A lower level of education’) is not correct .

- Ethnicity : Alzheimer’s incidence is lower in African Americans than in non-Hispanic whites , though they have a higher age-adjusted prevalence due to increased longevity . Alzheimer’s tends to be underdiagnosed in African American communities . Hispanics/Latinos also appear to have lower risk before late life , though cumulative incidence rises with age . Therefore , being African or Hispanic American is not associated with decreased risk ; it is closer to neutral (or sometimes lower) risk , and this option is ambiguous as presented .

- Marital status : Widowhood and marital loss are linked to higher risk of dementia , likely through increased psychosocial stress , isolation , lack of cognitive stimulation , and reduced healthcare access . Being married is generally associated with lower risk .

- City living vs rural : Some evidence suggests rural residence is associated with lower risk , whereas urban environments are linked to higher dementia incidence (e.g ., through cumulative stress , less green space , urban heat island effects , poor air quality , and neighborhood disadvantage). Thus , living in a city is not protective .

- Family history : A positive family history markedly increases risk , especially for autosomal dominant forms (e.g ., early-onset Alzheimer’s/FAD) and for late-onset disease overall . Having a family history is therefore not associated with decreased risk .

- Saturated fat intake : Data do not show a protective effect of higher saturated fat consumption ; healthier dietary patterns (e.g ., the MIND diet emphasizing fruits , vegetables , nuts , whole grains , and healthy fats , with less saturated fat) correlate with lower risk , but high saturated fat intake is not itself protective .

- Fish intake : Omega - 3 fatty acids in fish (particularly EPA and DHA) show mixed but generally supportive evidence for cognitive protection , with lower risk of cognitive decline and dementia observed in some longitudinal studies . Thus , eating fish is associated with decreased risk .

- Older age : Age is the strongest risk factor for Alzheimer’s ; the risk rises substantially after age 65 , with most cases occurring in individuals over 65 – 75 . Therefore , older age is not associated with a decreased risk .

- Smoking : Tobacco use is strongly harmful to cognition and increases dementia risk ; it is not protective .

- Alcohol : Light , regular alcohol consumption may be neuroprotective in some contexts (potentially via anti-inflammatory effects and improved cardiovascular risk) , but this is not uniformly protective , can be harmful with excess , and is not a primary or reliable protective factor for most people .

Evaluating the options , the factor most consistently associated with decreased risk of Alzheimer’s is a higher intake of fish , through its omega - 3 fatty acid content .

Correct answer: H. Eating fish

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<answer>
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